

National Coverage Determination (NCD)

FDG PET for Dementia and Neurodegenerative Diseases

220.6.13

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Tracking Information

Publication Number

100-3

Manual Section Number

220.6.13

Manual Section Title

FDG PET for Dementia and Neurodegenerative Diseases

Version Number**Effective Date of this Version**

04/03/2009

Implementation Date

10/30/2009

Description Information

Benefit Category

Diagnostic Tests (other)

Please Note: This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

Item/Service Description**A. General**

Medicare covers FDG Positron Emission Tomography (PET) scans for either the differential diagnosis of fronto-temporal dementia (FTD) and Alzheimer's disease (AD) under specific

requirements; OR, its use in a Centers for Medicare & Medicaid Services (CMS)-approved practical clinical trial focused on the utility of FDG PET in the diagnosis or treatment of dementing neurodegenerative diseases. Specific requirements for each indication are clarified below:

Indications and Limitations of Coverage

B. Nationally Covered Indications

1. FDG PET Requirements for Coverage in the Differential Diagnosis of AD and FTD

An FDG PET scan is considered reasonable and necessary in patients with a recent diagnosis of dementia and documented cognitive decline of at least 6 months, who meet diagnostic criteria for both AD and FTD. These patients have been evaluated for specific alternate neurodegenerative diseases or other causative factors, but the cause of the clinical symptoms remains uncertain.

The following additional conditions must be met before an FDG PET scan will be covered:

- a. The patient's onset, clinical presentation, or course of cognitive impairment is such that FTD is suspected as an alternative neurodegenerative cause of the cognitive decline. Specifically, symptoms such as social disinhibition, awkwardness, difficulties with language, or loss of executive function are more prominent early in the course of FTD than the memory loss typical of AD;
- b. The patient has had a comprehensive clinical evaluation (as defined by the American Academy of Neurology encompassing a medical history from the patient and a well-acquainted informant (including assessment of activities of daily living), physical and mental status examination (including formal documentation of cognitive decline occurring over at least 6 months) aided by cognitive scales or neuropsychological testing, laboratory tests, and structural imaging such as magnetic resonance imaging (MRI) or computed tomography (CT);
- c. The evaluation of the patient has been conducted by a physician experienced in the diagnosis and assessment of dementia;
- d. The evaluation of the patient did not clearly determine a specific neurodegenerative disease or other cause for the clinical symptoms, and information available through FDG PET is reasonably expected to help clarify the diagnosis between FTD and AD and help guide future treatment;
- e. The FDG PET scan is performed in a facility that has all the accreditation necessary to operate nuclear medicine equipment. The reading of the scan should be done by

an expert in nuclear medicine, radiology, neurology, or psychiatry, with experience interpreting such scans in the presence of dementia;

- f. A brain single photon emission computed tomography (SPECT) or FDG PET scan has not been obtained for the same indication. (The indication can be considered to be different in patients who exhibit important changes in scope or severity of cognitive decline, and meet all other qualifying criteria listed above and below (including the judgment that the likely diagnosis remains uncertain.) The results of a prior SPECT or FDG PET scan must have been inconclusive or, in the case of SPECT, difficult to interpret due to immature or inadequate technology. In these instances, an FDG PET scan may be covered after one year has passed from the time the first SPECT or FDG PET scan was performed.)
- g. The referring and billing provider(s) have documented the appropriate evaluation of the Medicare beneficiary. Providers should establish the medical necessity of an FDG PET scan by ensuring that the following information has been collected and is maintained in the beneficiary medical record:
 - Date of onset of symptoms;
 - Diagnosis of clinical syndrome (normal aging; mild cognitive impairment (MCI); mild, moderate or severe dementia);
 - Mini mental status exam (MMSE) or similar test score;
 - Presumptive cause (possible, probable, uncertain AD);
 - Any neuropsychological testing performed;
 - Results of any structural imaging (MRI or CT) performed;
 - Relevant laboratory tests (B12, thyroid hormone); and,
 - Number and name of prescribed medications.

The billing provider must furnish a copy of the FDG PET scan result for use by CMS and its Medicare Administrative Contractors upon request. These verification requirements are consistent with Federal requirements set forth in 42 Code of Federal Regulations, section 410.32 generally for diagnostic x-ray tests, diagnostic laboratory tests, and other tests. In summary, section 410.32 requires the billing physician and the referring physician to maintain information in the medical record of each patient to demonstrate medical necessity [410.32(d) (2)] and submit the information demonstrating medical necessity to CMS and/or its agents upon request [410.32(d)(3)(I)] (OMB number 0938-0685).

2. FDG PET Requirements for Coverage in the Context of a CMS-approved Practical Clinical Trial Utilizing a Specific Protocol to Demonstrate the Utility of FDG PET in the Diagnosis, and Treatment of Neurodegenerative Dementing Diseases

An FDG PET scan is considered reasonable and necessary in patients with MCI or early dementia (in clinical circumstances other than those specified in subparagraph 1) only in the context of an approved clinical trial that contains patient safeguards and protections to ensure proper administration, use and evaluation of the FDG PET scan.

The clinical trial must compare patients who do and do not receive an FDG PET scan and have as its goal to monitor, evaluate, and improve clinical outcomes. In addition, it must meet the following basic criteria:

- a. Written protocol on file;
- b. Institutional Review Board review and approval;
- c. Scientific review and approval by two or more qualified individuals who are not part of the research team; and,
- d. Certification that investigators have not been disqualified.

C. Nationally Non- Covered Indications

All other uses of FDG PET for patients with a presumptive diagnosis of dementia-causing neurodegenerative disease (e.g., possible or probable AD, clinically typical FTD, dementia of Lewy bodies, or Creutzfeld-Jacob disease) for which CMS has not specifically indicated coverage continue to be noncovered.

D. Other

Not applicable.

(This NCD last reviewed September 2004.)

Cross Reference

See NCD for [PET Scans](#) (§220.6).

[CED page](#)

Claims Processing Instructions

TN 1817 (Medicare Claims Processing)

TN 1833 (Medicare Claims Processing)

Transmittal Information

Transmittal Number

120

Coverage Transmittal Link

<https://www.cms.gov/Regulations-and-Guidance/Guidance/Transmittals/Downloads/R120NCD.pdf> 

Revision History

05/2010 - Transmittal 108, dated October 16, 2009, is being rescinded and replaced with Transmittal 120. This transmittal restores language that was inadvertently removed from section 220.6. This transmittal also updates section 220.6.17, per Transmittal 110, Change Request 6753, dated December 18, 2009. All other information remains the same. ([TN 120](#) ) (CR 6632)

10/2009 - This change request rescinds and replaces Transmittal 106, dated September 18, 2009. The effective date has been changed to April 3, 2009 and the implementation date has been changed to October 30, 2009. Business Requirements (BR) 6632.6.1 and 6632.6.2 have been revised to clarify that they are subsets of BR 6632.6 and are specific to CED. All other information remains the same. ([TN 108](#) ) (CR 6632)

04/2005 - Added "PET (FDG) for" to NCD title. Effective date 01/28/05. Implementation date 04/18/05. ([TN 31](#) ) (CR 3741)

10/2004 - Covered use of FDG-PET Scans for differential diagnosis of fronto-temporal dementia and Alzheimer's disease under specific requirements; OR its use in CMS-approved practical clinical trial focused on utility of FDG-PET in diagnosis or treatment of dementing neurodegenerative diseases. Effective date 09/15/2004. Implementation date 10/04/2004. ([TN 24](#) ) (CR 3426)

National Coverage Analyses (NCAs)

This NCD has been or is currently being reviewed under the National Coverage Determination process. The following are existing associations with NCAs, from the National Coverage Analyses database.

- [Original Consideration for Positron Emission Tomography \(FDG\) for Alzheimer's Disease/Dementia \(CAG-00088N\)](#) 
- [First reconsideration for Positron Emission Tomography \(FDG\) and Other Neuroimaging Devices for Suspected Dementia \(CAG-00088R\)](#) 

Additional Information

Other Versions

Title	Version	Effective Between	
FDG PET for Dementia and Neurodegenerative Diseases	3	04/03/2009 - N/A	You are here
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Dementia and Neurodegenerative Diseases	1	09/15/2004 - 01/28/2005	View